

Patient: MUSLIM, MUSTAMIRR (AM, 09/10/24) (Male)
 39 NIAGARA LANE
 WILLINGBORO, NJ 08046 (201)887-9347
 PATRICIAANN1G@GMAIL.COM
DOB: 05/27/1963 (62)
Race: Patient Declined
Language: English
Ethnicity: Patient Declined
Encounter ID: 214510265
Primary Ins: New Jersey
 Manufacturers NJM -
 Auto Only E1118

Location: Elite Specialty Care- Clifton Hq
 855 VALLEY RD
 clifton, NJ, 07013-3850
 (973)305-8830
Provider: DR. BADER ELDER, DO
Referring: Fotiou -Trenton, William

Subjective

Chief Complaint:
10/01/25

Patient presents today for re-evaluation of cervical and lumbar spine. The patient is following post epidural steroid injection.

Procedure history:

- Rhizotomy lumbar performed on 9/25/25
- Rhizotomy lumbar performed on 9/11/25
- Cervical epidural steroid injection (CESI) performed on: 4/24/25
- LFB performed on: 7/31/25

Pain characteristics since injections:

- Cervical pain: Patient reports 75% relief after CESI. Cervical pain now rated 2/10, described as numbness on the right side, without radiating pain.
- Lumbar pain: Patient reports 95% relief after LESI. Lumbar pain rated 3/10, without radiation. Improvement noted after injection.
- Patient reports very good progress and recovery following the procedure.

Therapy and Interventions:

- The patient is not currently participating in physical therapy / chiropractic care for this pain.
- Medications: The patient takes NSAIDs on a scheduled basis as needed, with symptomatic improvement during the effective duration of the medication.

08/25/25

Patient presents today for re-evaluation of cervical and lumbar spine. The patient is following post epidural steroid injection.

Procedure history:

- LFB 2 on 7/31/25 post op 100% relief short term

Pain characteristics since injections:

- Cervical pain now rated 3/10, without radiation.
- Lumbar pain: Patient reports 46% relief after LESI. However, pain returned approximately 2 weeks post-injection and has been gradually increasing in intensity. Lumbar pain rated 4/10, without radiation / with radiation pain down to bilateral lower extremities, especially the ___ leg. Improvement noted after injection.
- Patient reports very good progress and recovery following the steroid injection. / Patient reports increased pain 2–3 days / weeks / month after the relief steroid injection.

Therapy and Interventions:

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- The patient is not currently participating in physical therapy / chiropractic care, patient finished his therapy.
- Medications: The patient takes NSAIDs on a scheduled basis as needed, with symptomatic improvement during the effective duration of the medication.

07/08/25

Patient is here for a follow-up after a LMBB injection performed on June 15th of this year. Patient states he has approximately 80% relief

Patient reports the injection provided 80% relief initially for a few days but the pain has returned since then.

Neck: 6/10

Back: 6/10

Patient reports intermittent tingling and shooting pain down the right arm, likely radiating from the neck. Denies any radiating pain from the back to the legs. No other extremity pain noted. Localized back pain is painful.

Physical therapy and chiropractic care are currently not ongoing.

Patient is here for a post op LESI on 4/10/25 and Cesi 4/24/25

Patient states that he is experiencing around 70% relief from the LESI and around 50% relief from the CESI

Neck: 6/10 patient states that the pain feels more muscular and stiffness

Back: 7/10

Chiro is no longer ongoing

This 61 year old male patient presents with complaints of Neck and back pain in the axial plane of the cervical and lumbar spine. Neck pain is 7/10 and the back pain is 8/10.

Patient is not experiencing any radiating pain going down the arms or legs

The pain is associated with an auto accident where he was a passenger struck by a car on his left side by a commercial vehicle. Airbags did not deploy. He reports no loss of consciousness. No Immediate treatment was rendered at the scene for neck and back injuries. He admits to neck or back pain complaints prior to the most recent incident, due to an accident in 2022 The patient has experienced this pain since 09/10/2024

The severity of pain is moderate to severe.

The pain character is described as sharp, dull and stiffness.

It occurs constantly, with varying intensity.

Pain Rating: (0 = no pain at all. 10 = worst pain you can imagine)

Least your pain has been in the past week: 6/10

Average pain during the week: 7/10

Worst pain has been in past week: 8

Pain interference: (0 = pain does not interfere at all. 10 = pain completely limits or prevents the item)

Activity: 3/4

Sleep: 5/10

Modifying factors: The pain is increased by sitting, driving, standing, walking, physical activity, early morning hours, forward flexion and getting up out of a seated position and the pain is relieved by rest, changing positions and hot fomentations.

Associated signs and symptoms include tingling, numbness and muscle spasm and tightness.

Previous treatments:

Chiropractic manipulation is still ongoing.

Medication List

Date	Medication	Sig	#	Refill	Status
07/08/2025	Celebrex 200 mg capsule	Take 1 capsule by mouth twice a day as needed (PRN); drink plenty of water and take with food	60	0	Active
07/08/2025	Fexmid 7.5 mg tablet	Take 1 tablet by mouth three times a day as needed (PRN); do not drink or drive while utilizing	90	0	Active
07/08/2025	pantoprazole 20 mg tablet, delayed release	Take 1 tablet by mouth twice a day as needed (PRN); take with NSAIDS for GI PPX	60	0	Active
07/08/2025	Zylorel 4 %-1 % topical patch	Apply 1-2 patch on the skin daily as needed (PRN); apply to neck and/or back region 12 hours on and 12 hours off	60	0	Active
05/13/2025	cyclobenzaprine 10 mg tablet	Take 1 tablet by mouth three times a day as needed (PRN); do not drink or drive while utilizing	90	0	Active

Allergies

The patient has no known allergies.

Social Hx

Tobacco	Usage Status	Last Used	Daily Usage	Packaging	Years	Life History	Cessation Attempts	Last Cessation Rx
Cigarettes	Never used tobacco			Packs				

Alcohol : Patient has no history of alcohol use.

Surgical Hx

Patient has no history of surgery.

Review of Systems

Respiratory: negative for cough dyspnea wheezing

Cardiovascular: negative for chest pain and irregular heartbeat/palpitations

Gastrointestinal: negative for diarrhea and vomiting

Genitourinary: Negative for dysuria hematuria and polyuria.

Dermatologic: Negative for pruitus and rash

General: No weight change, no fatigue, no fevers, no chills, no change in strength. Head: No vertigo, no injury. Eyes: Normal vision, no diplopia, no tearing, no ocular pain. ENT: No change in hearing, no tinnitus, no bleeding, no vertigo, no epistaxis. Chest: No dyspnea, no wheezing, no hemoptysis, no chronic cough. Heart: No chest pains, no palpitations. GI: No change in appetite, no dysphagia, no abdominal pains. GU: No urinary urgency, no dysuria, no hematuria, no change in nature of urine. Neuro: No syncopal episode or seizures. Psych: No depressive symptoms, no

changes in sleep habits, no changes in thought content. All other systems are negative.

MSK: +HPI

Respiratory: negative for cough dyspnea wheezing

Cardiovascular: negative for chest pain and irregular heartbeat/palpitations

Gastrointestinal: negative for diarrhea and vomiting

Genitourinary: Negative for dysuria hematuria and polyuria.

Dermatologic: Negative for pruitus and rash

General: No weight change, no fatigue, no fevers, no chills, no change in strength. Head: No vertigo, no injury. Eyes: Normal vision, no diplopia, no tearing, no ocular pain. ENT: No change in hearing, no tinnitus, no bleeding, no vertigo, no epistaxis. Chest: No dyspnea, no wheezing, no hemoptysis, no chronic cough. Heart: No chest pains, no palpitations. GI: No change in appetite, no dysphagia, no abdominal pains. GU: No urinary urgency, no dysuria, no hematuria, no change in nature of urine. Neuro: No syncopal episode or seizures. Psych: No depressive symptoms, no changes in sleep habits, no changes in thought content. All other systems are negative.

MSK: +HPI

Objective

Physical Exam

Today;

Beared gentlement, glasses, hoody, military pants

GENERAL: The patient is resting in a seated position in no acute distress. VSS. Appears stated age. Pleasant. Provides coherent history. The patient did not demonstrate any pain behavior, symptom magnification or drug seeking behavior.

PSYCHIATRIC: Mood stable. Affect appropriate. Language processing intact. Speech of normal rate. No psychomotor slowing noted. The patient demonstrates no signs of hypersomnolence, sedation or confusion.

SKIN: No rashes dystrophic changes.

NEUROLOGICAL: Alert and oriented. No aphasia with fluency and comprehension intact. No tremors.

HEENT: Normocephalic. Atraumatic. Pupils are appropriate size for room lighting.

CARDIOVASCULAR: No visible lower extremity edema. Bilateral radial pulses intact.

RESPIRATORY: Nonlabored breathing pattern. No accessory muscle use. No audible wheeze. No cough. Symmetric chest wall excursion.

UPPER EXTREMITY NEUROLOGIC: Muscle strength is 5/5 in all major proximal and distal muscle groups of the upper extremities. Sensation is grossly intact in all major dermatomes of the upper extremities.

LOWER EXTREMITY NEUROLOGIC: Muscle strength is 5/5 in all major proximal and distal muscle groups of the lower extremities. Sensation is grossly intact in all major dermatomes of the lower extremities.

MUSCULOSKELETAL: The patient is able to rise from a seated to a standing position without significant difficulty and delay. Patient has stable gait, no ambulatory assistance.

Cervical spine: Tenderness to palpation is noted in the bilateral cervical paraspinal musculature. Taut muscle bands noted bilaterally. Limited cervical active ROM of rotation and side bending. Spurling's test is positive bilaterally. Cervical distraction test maneuver noted.

Lumbar spine: Lumbar facet loading is positive bilaterally. Tenderness to palpation is noted in the bilateral lumbar paraspinal musculature. Lumbar flexion and extension are limited due to pain. Lumbar facet loading is positive bilaterally. Straight leg raising is positive bilaterally. Positive Slump test bilaterally.

Sacroiliac: negative

Assessment

Diagnosis

M5416 Radiculopathy, lumbar region
M5412 Radiculopathy, cervical region
M62838 Other muscle spasm
M4697 Unspecified inflammatory spondylopathy, lumbosacral region
M4692 Unspecified inflammatory spondylopathy, cervical region

Disposition

Code	Description
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Plan

Procedures

Description	Code	Units	Modifiers	Comments
Established Patient E/m: History, Exam, Low-Level Decision Making	99213	1 UN		

Care Plan

Plan:

1) LEFT RFA Lumbar L4, 5, and S1-- complete asap
RIGHT RFA Lumbar L4,5 and S1 first-- completed Right side is feeling very well.
--Right side completed twice

LMBB #2 L4,5 and S1 under fluoro order - successful
LMBB #1 L4, 5, S1 under fluoro bialteral --80% relief relief
LESI L4/5 w TPI -70% relief, pain tapered back
2) Monitor cervical might need Cmbb
CESI w/ TPI - 60% relief, pain is tapering back.
3) Completed Chiro

Patient seen and evaluated via telemedicine. Patient presents today after erroneously having RFA repeated on the right side unfortunately. We discussed this with patient. No serious effects or side effects have been reported by patient. Patient will continue with appropriate conservative management with home exercises. Patient would like to complete his RFA on the left side.

Prev via Newark office. Patient presents today for further evaluation after completing LMBB#2. Patient did very well, we will continue with lumbar rhizotomy therapy at this time. First right side, followed by left side after.

Prev, via Trenton office. Patient underwent lumbar medial branch block #1. Patient states he has greater than 80% relief. Patient doing much better after the injection. Patient states this worked better than lumbar epidural. Patient neck pain is still bothering him more localized pain. We will monitor his cervical spine. At this time we will continue treating as lumbar. The patient has success with lumbar medial branch block #2 we will going to rhizotomy successfully. Patient requesting medication refill. I will send medications accordingly.

The combined use of NSAIDs, muscle relaxers, and topical lidocaine patches provides a comprehensive, multimodal approach to managing pain syndromes. This regimen addresses inflammation, muscle spasms, and localized nerve pain, offering effective and well-tolerated relief for a wide range of conditions. This is non-opioid and conservative/safe medical approach to pain management.

Previously, via patient presents for reevaluation after cervical epidural steroid injection. Patient initially had significant amount of relief but unfortunately the pain has tapered right back up. Overall these procedures were successful and we would have liked. Today's exam more pertinent for facet syndrome. We will focus on his lumbar region at this time. He is continuing chiro. Medication will be sent today.

Based on the patient's symptoms and exam findings, I discussed possible benefits of lumbar facet nerve blocks as a diagnostic procedure to help determine the source of the pain and whether or not a rhizotomy may eventually be of benefit. Conservative treatments including therapy and medications have only been of limited benefit.

Prevoiusly, the patient is a 61 yo M with PMHX of HTN consulted to my service presents today for further evaluation of cervical and lumbar spine. Patient has cervical pain which is causing numbness and tingling in her hands states she never had that prior to the accident. Patient's low back pain does cause radiating symptoms intermittently. Patient has done significant amount of chiro care without any significant improvement at this time. We reviewed patient's MRIS in great detail. We utilized an anatomical model while

reviewing. The patient had the opportunity to ask questions and all questions were answered to patient's satisfactory. Patient exhibited an understanding of their Mri findings. Patient still has pain and limited function of the cervical and lumbar region.

Electronically Signed and Reviewed by ELDER, BADER 10/01/2025 14:29:39